

ROYAL BROMPTON HOSPITAL

Department of Cardiology — Outpatient Clinic

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PRE-TRAVEL CARDIOLOGY ASSESSMENT

Date of consultation: 18 January 2026 | Ref: RBH-CARD-OUT-2026-00847

PATIENT DETAILS

Patient Name:	Margaret Elaine CHEN
Date of Birth:	14 March 1967
NHS Number:	943 847 2910
Address:	42 Pembroke Road, Kensington, London W8 6NX
Passport:	GB-982451673
Referred by:	Dr. James Whitfield, GP, The Kensington Practice

REASON FOR CONSULTATION

Mrs. Chen was referred for pre-travel cardiology assessment ahead of a planned three-week holiday to **Vietnam**, departing 22 February 2026. She has a history of essential hypertension managed with medication and her GP wished to confirm fitness for long-haul air travel.

CLINICAL EXAMINATION

Blood pressure: **142/91 mmHg** (seated, right arm, after 5 min rest). Repeat reading: 138/88 mmHg. Heart rate: 74 bpm, regular. BMI: 26.3.

Cardiovascular examination: Heart sounds I + II normal. No murmurs. No peripheral oedema. JVP not elevated. Carotid pulses normal, no bruits. Lung fields clear bilaterally.

INVESTIGATIONS

Investigation	Result	Reference	Comment
Resting ECG	Normal sinus rhythm, 74 bpm	—	No ST changes, no LVH
Echocardiogram	EF 58%, mild diastolic dysfunction	EF > 55%	Grade I diastolic dysfunction
Exercise stress test	Completed 9 min Bruce protocol	—	No chest pain, no ST changes
HbA1c	5.4%	< 5.7%	Normal
Fasting lipids	Total 219, LDL 138 mg/dL	TC<200, LDL<100	Borderline — lifestyle advice given

CURRENT MEDICATION

1. **Amlodipine 5mg** once daily (morning)

2. **Losartan 50mg** once daily (evening)

No other regular medications. No allergies documented.

Note: Given the borderline BP readings today, I have discussed with Mrs. Chen the option of increasing Amlodipine to 10mg. She prefers to continue at current dose and monitor. I have advised her to bring a home BP monitor on her trip and to seek medical attention if readings consistently exceed 150/95.

FITNESS TO FLY ASSESSMENT

OPINION: FIT FOR AIR TRAVEL

Mrs. Chen is a 58-year-old woman with well-controlled essential hypertension and no evidence of end-organ damage. Her exercise capacity is good (9 minutes Bruce protocol without symptoms or ECG changes). Echocardiographic function is preserved. She is fit for long-haul air travel.

I have advised the following precautions for travel: adequate hydration, regular mobilisation during flights, compression stockings for DVT prevention, continue all regular medications, and ensure travel insurance covers her pre-existing hypertension.

FOLLOW-UP

Routine follow-up appointment booked for **20 March 2026** at this clinic to review post-travel BP readings and discuss lipid management strategy. If Mrs. Chen experiences any chest pain, significant breathlessness, or sustained BP >160/100 during travel, she should seek immediate medical attention locally.

Dr. Richard Langford, BSc MB ChB FRCP

Consultant Cardiologist, Royal Brompton Hospital

Honorary Senior Lecturer, Imperial College London

GMC Registration: 4187293

CC: Dr. James Whitfield, The Kensington Practice, 12 Abingdon Road, London W8 6AF